

26STCV17591 26STCV17591

County: los-angeles

Division: Civil Law and Motion

Department: 14

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Case Number: 26STCV17591 Hearing Date: August 14, 2026 Dept: 14

SUPERIOR COURT OF THE STATE OF
CALIFORNIA¿¿

FOR THE COUNTY OF LOS ANGELES

DEPARTMENT 14¿

¿

CAROLE E. GORDON,

Plaintiff,¿

v.

ALTICOR INC., et al.,

¿

Defendants.¿¿

Case No. 26STCV17591

¿¿¿¿

Hearing Date: August 14, 2026

Time:

9:00 a.m.

¿ [TENTATIVE] ORDER

RE:¿

PLAINTIFF'S MOTION FOR TRIAL PREFERENCE

I.

Background

On June 4, 2026, Plaintiff Carole E. Gordon filed her complaint for personal injury alleging she developed malignant mesothelioma (with sarcomatoid features) from non-occupational exposure to asbestos and asbestos-containing cosmetic products. On July 21, 2026, Ms. Gordon filed this amended motion for trial preference under Code of Civil Procedure section 36, subdivision (a). Ms. Gordon argues that she is 85-years old and her health is such that preference is necessary to prevent prejudicing her interests in the litigation. Ms. Gordon requests a trial be set 120 days from the date of the hearing on this matter.

On August 3, 2026, Defendant Merle Norman Cosmetics, Inc. ("Merle Norman") filed an opposition. Merle Norman opposed the motion on the grounds that Ms. Gordon has not presented sufficient evidence that her health is such that preference is necessary to prevent prejudicing Ms. Gordon's interests in the litigation. Alternatively, Merle Norman argues that if the court were to grant preference, specific conditions should be imposed, such as for trial date timing and discovery considerations.

Ms. Gordon replied on August 7, 2026. She argued that preference under section 36, subdivision (a) is mandatory and Merle Norman mistakenly contended that preference is discretionary. Ms. Gordon further argues that Merle Norman failed to provide evidence that Ms. Gordon's health is such that a preference is not necessary to prevent prejudice.

Ms. Gordon presented sufficient evidence to show that her health is such that preference is necessary to prevent prejudicing her interests in the litigation. Therefore, Ms. Gordon's motion for trial preference is granted. Trial is set for Monday, December 7, 2026. The Final Status conference is set for Monday, November 16, 2026. A status conference for the proposed trial setting order is set for Wednesday, August 26, 2026.

II.

Discussion

A.

Legal Standards

A party who is over 70 years old may petition the court for a preference for trial date, which the court shall grant if it makes both of the following findings: (1) the party has a substantial interest in the action as a whole; and (2) the health of the party is such that a preference is necessary to prevent prejudicing the party's interest in the litigation. (Code Civ. Proc., § 36, subd. (a).) An affidavit submitted in support of a motion for preference under this subdivision may be signed by the attorney for the party seeking preference based upon information and belief as to the medical diagnosis and prognosis of any party. (Id., § 36.5.)

"Upon the granting of such a motion for preference, the court shall set the matter for trial not more than 120 days from that date and there shall be no continuance beyond 120 days from the granting of the motion for preference except for physical disability of a party or a party's attorney, or upon a showing of good cause stated in the record."

(Id., § 36, subd. (f).)

"Any continuance shall be for no more than 15 days and no more than one continuance for physical disability may be granted to any party." (Ibid.)

B.

Analysis

Ms. Gordon is eighty-five years old and was diagnosed with malignant mesothelioma of the pleura, sarcomatoid type, in February 2026. (Moline Decl., ¶¶ 4, 6, 12-13, Exh. C.) Ms. Gordon has a history of "left-breast ductal carcinoma in situ" that was treated with "bilateral mastectomies in 2013." (Id., ¶ 4.) On January 26, 2026, a PET/CT scan demonstrated "FDG-avid left cardiophrenic lymph nodes (the largest measuring approximately 5 x 10 mm, with an SUV max of 5.4); a small-to-moderate left pleural effusion with an intensely FDG-avid soft-tissue pleural nodule measuring approximately 17 x 11 mm (SUV max 10.6); scattered left fissural nodularity; and focal moderate-to-intense FDG activity within the sacrum without a CT correlate (SUV max 4)." (Id., ¶ 5, Exh. B, Tab 1.) On February 14, 2026, Ms.

Gordon's pathologic examination (left pleural biopsy) was finalized, showing: "malignant mesothelioma with sarcomatoid features (at least 20%), and the left pleural mass biopsy demonstrated malignant mesothelioma with transitional features." (Id., ¶ 6, Exhs. B, Tab 4, C ["mesotheliomas with transitional features are considered a subset of sarcomatoid mesotheliomas.1 Immunohistochemical staining of the tumor cells was positive for calretinin, WT-1, D2 40, CK5/6, and GATA3, and negative for claudin-4, BerEP4, and MOC31, with loss of BAP1 expression."].)

Ms. Gordon was found to not be a surgical candidate on February 26, 2026. (Id., ¶ 7 ["Progression of her disease, therefore, cannot be slowed through surgical intervention."].) On March 11, 2026, Ms. Gordon began non-curative "first-line systematic immunotherapy with nivolumab and ipilimumab." (Id., ¶ 8, Exh. B, Tab 6.) Ms. Gordon received additional immunotherapy on April 1, April 22, and May 13, 2026, and continues to receive said therapy about every three weeks. (Id., ¶ 9, Exh. B, Tabs 7-8.) Ms. Gordon also has an indwelling tunneled pleural catheter to help manage her recurrent malignant left pleural effusion, which is drained weekly. (Ibid.)

On May 29, 2026, Ms. Gordon had a follow-up PET/CT scan. (Id., ¶ 10.) The study showed "a near-complete resolution of the previously intensely FDG-avid pleural nodule, with only a residual subtle 4 mm soft-tissue density below the threshold for FDG evaluation, and an interval resolution of the previously prominent FDG-avid left cardiophrenic lymph nodes." (Ibid.) The scan "also showed persistent left fissural nodularity concerning for pleural carcinomatosis and metastatic disease; persistent intense focal FDG avidity in the sacrum without a definite CT correlate, for which osseous metastatic disease was not excluded; and a subcentimeter left axillary lymph node with a mild interval increase in FDG uptake." (Ibid.) On June 2, 2026, Ms. Gordon's oncologist determined that the imaging "showed a good response after four doses of immunotherapy." (Id., ¶ 11, Exh. B, Tab 9.) However, because Ms. Gordon developed "immune-related adverse effects of treatment," like pneumonitis and pruritic dermatitis, the ipilimumab was discontinued. (Ibid.)

As of June 2, 2026, Ms. Gordon's Eastern Cooperative Oncology Group performance status was 2 and her diagnoses has been characterized as "carrying a high risk of complication,

morbidity, or mortality.” (Ibid.) Dr. Moline opines that “there is substantial medical doubt that Ms. Gordon will survive beyond six (6) months from the date of this declaration.” (Id., ¶ 14.)

Merle Norman is the only Defendant to oppose Ms. Gordon’s motion. In opposition, it argues that Ms. Gordon has failed to meet the statute’s requirements:

“As shown above, the last two purported medical records of Ms. Gordon establish: (1) Plaintiff is ‘doing well overall,’ is ‘still able to perform ADLs’ and goes ‘out with friends and do other activities; (2) had a good response to chemotherapy; (3) that her PET/CT of May 29, 2026 shows ‘Near complete resolution of prior intensely FDG avid pleural nodule with a residual subtle 4 mm soft tissue density below the threshold for FDG evaluation’; and (4) Plaintiff’s physical exam was normal, including plaintiff’s respiratory system, Plaintiff was to resume infusions in mid-June and will have her next scan in August.

Importantly, what Plaintiff fails to present to the court is any actual medical evidence (or really any information whatsoever) regarding: 1) Whether Plaintiff’s symptoms and/or disease have been getting worse; 2) Plaintiff’s current diagnosis.”

(Opp., at p. 4 [emphasis in original].)

Merle Norman further argues that the declarations of Ms. Gordon’s attorney and non-treating expert doctor “fail to establish that Plaintiff’s health is in immediate peril or that her present condition puts her at imminent risk of being unable to participate meaningfully at trial in this matter.” (Ibid.) Pointing to Ms. Gordon’s scheduled PET/CT scan in August, Merle Norman argues the motion should be continued to allow for the results to be shared. (Id., at p. 5.)

The evidence before the court is sufficient to establish that Ms. Gordon is entitled to a trial preference under Code of Civil Procedure section 36, subdivision (a). There is no dispute that Ms. Gordon’s age is above the threshold required under the

statute, or that she has a substantial interest in the litigation. The only issue in dispute is whether her health is such that her interest in the litigation is likely to be prejudiced in the absence of trial preference.

The court finds that the evidence presented is sufficient to satisfy this burden. The declarations of Mr. Horowitz, Dr. Moline, and attached medical records are sufficient to show that Ms. Gordon's current symptoms and the likely progression of her symptoms given the absence of viable treatment will impact her testimonial capacities to her prejudice. For instance, Dr. Moline states that due to the aggressive nature of Ms. Gordon's mesothelioma and her increased age, her physical strength is diminished. (Moline Decl. ¶ 12 ["Although her most recent imaging reflects a favorable radiographic response of the dominant pleural nodule, responses in sarcomatoid mesothelioma are characteristically shallow and short-lived. Her residual and probable metastatic disease, limited tolerance of treatment, advanced age, and diminished performance status leave her with little physiologic reserve. Her symptoms are expected to progress as her disease advances, ultimately resulting in her death."].) This, paired with the evidence that Ms. Gordon lacks viable treatment options and the extremely aggressive nature of malignant mesothelioma of the pleura, sarcomatoid type, is sufficient to persuade the court that her health is such that Ms. Gordon's interest in the litigation will be prejudiced without trial preference.

Moreover, Ms. Gordon was not required to present a declaration from a treating physician. (Code Civ. Proc., § 36.5 ["An affidavit submitted in support of a motion for preference under subdivision (a) of Section 36 may be signed by the attorney for the party seeking preference based upon information and belief as to the medical diagnosis and prognosis of any party. The affidavit is not admissible for any purpose other than a motion for preference under subdivision (a) of Section 36."].) Dr. Moline's and counsel's declarations are sufficient. Accordingly, Ms. Gordon's motion for trial preference is granted.

Merle Norman's opposition also requests that the court impose certain conditions as part of the trial setting order including, but not limited to, shortening the motion for summary judgment

and/or adjudication notice period, ordering third-party witnesses' immediate availability for depositions, and ordering Ms. Gordon to provide any outstanding relevant records. The court lacks the power to shorten notice for summary judgment. (McMahon v. Superior Court (2003) 106 Cal.app.4th 112, 116.) Merle Norman cites no authority for the proposition that the court can impose such conditions upon the parties as part of an order granting trial setting preference. The court expects the parties to meet and confer regarding the appropriate conditions for a proposed trial setting order. Ms. Gordon's motion indicates that she is willing to agree to certain stipulations and to extend all discovery deadlines past the statutory cut-offs. (Motion at p. 8; Horowitz Decl., ¶¶ 9-10.) If there are disputes as to the appropriate conditions, the court will rule on these disputes at the status conference regarding the trial setting order.

III.

Conclusion

Ms.

Gordon's motion for trial preference is granted. The parties are to meet and confer regarding appropriate conditions for a trial setting order. The court sets trial for Monday, December 7, 2026, at 9:00 A.M. The Final Status conference is set for Monday, November 16, 2026 at 9:00 A.M. A status conference for the proposed trial setting order is set for Wednesday August 26, 2026 at 9:00 A.M. Ms. Gordon is ordered to give notice.